1. Time plan per question (18 min)

Split

- 2 min: read stem twice, underline every task word (discuss, compare, outline, evidence)
- 1 min: plan headings matching the parts and their marks
- 13 min: write
- 2 min: check every part answered, add a summary line

Paper level

- Answer easiest first, never leave a question blank: the first marks of each question are the easiest
- Stop at 18 min; return if time remains

Key point: Marks follow the parts: a blank part costs more than a short one.

2. Universal answer frame

Seven headings

- **1. Interpretation**: what is going on, severity, numbers
- **2. Differential**: most likely first, life-threatening not to miss
- **3. Immediate management**: ABC, resuscitation, specific first actions
- **4. Definitive / ongoing care**: targets, drugs with doses, monitoring
- **5. Evidence**: trial name + year + one-line result
- **6. Controversy and emerging**: what is debated, what is new
- **7. Ethics and communication**: family, consent, limits of care

Key point: Use the frame for every scenario: examiners recognise the structure.

3. Writing style that scores

Do

- Headings and short bullet points
- Numbers and doses (MAP 65, Vt 6 mL/kg PBW, lorazepam 0.1 mg/kg)
- Name trials with year and result
- Prioritise: "first …, then …"
- Answer the exact question asked (if it says compare, use a table)

Don't

- Long prose paragraphs, repeating the stem
- Lists without explanation for "discuss"
- Abbreviations without first spelling them out

Key point: One idea per line; one mark per idea.

4. Command words

Meanings

- **Outline**: list main points briefly
- **Describe**: what, how, with detail
- **Discuss**: points for and against, evidence, conclusion
- **Compare and contrast**: similarities and differences, ideally a table
- **Justify / rationale**: why, with evidence
- **Evaluate**: strengths, weaknesses, verdict

Key point: The command word tells you how to write the part.

5. Evidence: how to quote a trial

Template

- Name (year), population, intervention vs control, main result, limitation
- e.g. "PROSEVA (2013): severe ARDS P/F < 150, prone ≥ 16 h/day → 28-day mortality 16% vs 33%; experienced centres."

When unsure

- Say the direction of evidence: "large RCTs show no mortality benefit"
- Quote guidelines: SSC 2021, ESICM 2023, BTF 2016, ATS 2015

Key point: Two well-quoted trials beat five vague ones.

6. Example opening: septic shock

First lines

- "68-year-old diabetic with septic shock from faecal peritonitis: vasopressor-dependent, MAP < 65 despite 30 mL/kg; high risk of death (> 40%)."
- "Immediate priorities run in parallel: airway and oxygen, MAP 65 with noradrenaline, antibiotics within one hour, source control within 6–12 hours."

Closing line

- "Daily review: de-escalate antibiotics, stop steroids when off vasopressors, discuss goals with family."

Key point: A strong first and last line frame the whole answer.